

Acid Lullabies

How LSD Reversed My Chronic Insomnia

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From June 2022 to December 2024 I consumed approximately 57,000 μ g of LSD across approximately 123 dosing sessions, making for an average dose of 500 μ g taken on average of every five days. This was, mostly, in an attempt to finally sleep for once in my life. This is the story of how I discovered LSD reverses my insomnia for a short period of time, but also how after extended use, it may have reversed it once and for all. The story of this discovery actually begins in May of 2019, but it all requires the preface of what brought me here in the first place, which is the inclusion of my medical history, as well as any potentially applicable genetic data.

MEDICAL HISTORY

I was born and raised in the Salt Lake Valley in February of 1989, making me 36 years old as of typing this. According to DNA tests I am a mixture of Northwestern European (French-German), British & Irish, Greek and Balkan, Macedonian, Albanian, and Scandinavian descents. I am six foot, typically around 170-180 pounds, dark hair, with blue/green eyes.

I had my first out-of body experience when I was one year old. Beginning around age five I began experiencing episodes of both derealization and depersonalization. Around age seven I began experiencing sleep disturbances, including trouble initially falling asleep often due to restlessness, vivid recurring dreams which were also out-of-body experiences, (these lasted until age eighteen), and sleep paralysis episodes. Around this age I began experiencing symptoms of what I have since come to know as Visual Snow Syndrome, but which had gone undiagnosed my entire life because I never reported it. I didn't think much of it was odd until I started doing psychedelics as an adult and all that was happening was a bit more enhanced version of much of what I had already been experiencing naturally since age seven.

Since late-2003 I have been in widespread chronic pain from head to toe. This brought on chronic insomnia. It may be said that from the younger age I also suffered from insomnia due to those disturbances, but it was more a transient insomnia. I have been tested for all variations of arthritic and osteo concerns, Lyme disease, Lupus, Multiple Sclerosis (only non-neurological symptomology testing), Crohn's disease, Marfan's Syndrome, and likely several others I don't remember. I have also been prescribed around two dozen different medications from SSRIs, SNRIs, Muscle Relaxers, and Opiates, probably other forms I don't recall, and a handful of physical or restorative therapies. So many options that I've joked for some time now that in the end I think the only thing I didn't ever try was acupuncture.

The final assessment I was given was the inconclusive and enigmatic label of juvenile fibromyalgia. It was odd to everyone that a thirteen to fifteen year old boy was reporting symptoms most often seen in women approximately aged 38–55, but it was the only answer we had. I passed the known pressure point and other physical exams which existed at the time. SSRIs, SNRIs, and Muscle Relaxers were a mixture of ineffective, or began with side-effects that made things worse, so I stopped using early. Opiates were completely ineffective. The only one my body ever responded to, other than morphine via express-direct hospital delivery, was Percocet, but it began to fatigue and tire me, and I had to keep increasing the dose, so I stopped. I mostly accepted my fate and lived in the pain, trying things here and there as opportunity seemed hopeful or at least worth an attempt.

In 2008 I had a sleep paralysis episode which caused an intense 9-month spike in the level of the pain I experienced. This also brought about an eight year episode of Musicophilia. I still experience it on occasion, most often as I am waking up. My current medical provider speculates this could be related to something like Todd's paralysis, but I have yet to undergo those evaluations.

In 2018 I had an encounter with serotonin syndrome. A scary mixture of Ondansetron and Cannabis. I have avoided Ondansetron and any medications that could potentially lead to that again, as in 2017 I began using medical cannabis for the chronic pain, and it works better than anything with no ill side-effects as long as I have the correct prescription.

In 2020 I received a test result from a lab associated with the Illinois College of Medicine at Chicago which was seeking to find indicators for those reporting diagnosis of fibromyalgia. They reported that I had a strongly confirmable case with an 89/100 score for fibromyalgia as according to their research criteria. The result involved the revelation of a Peripheral Blood Mononuclear Cell dysfunction wherein I struggle to produce normal quantities of chemokine's and cytokines. If this is true, I personally find this to be the likely reason for my overall underlying body ache, which makes it feel as if I always have the same aches and pains that often accompany having the flu—yet I rarely get the flu, or any similar sickness. When I do this pre-existing body ache is simply amplified. Perhaps it's that I don't really notice the minor cases, or it makes me more often a carrier. Either way, it doesn't seem to account for all the other aches and pains, which are things like a constant diffuse rib pain, and almost every pain sensation appearing literally anywhere in the body at random; sharp, shooting, burning, dull, pricking, neuropathic, sporadic numbness of digits and extremities, and rare occasions of pain sensitivity to fabric or another humans touch.

In 2022 I finally came to the realization, and it has since been made conclusive, that I have Celiac's disease. I either didn't report stomach discomfort when I was younger, or it wasn't often or dire enough to warrant much concern. Though even just two years since my diet change when I accidentally ingest gluten my response is more often a cluster headache or migraine, and intensified body ache, with maybe a later onset of stomach or intestinal discomfort. Those abdominal

discomforts definitely comes after the ingestion of dairy, particularly butter and heavy creams. Lactose intolerance often accompanies Celiac's.

LSD INTERACTION

As I said in the beginning, the story really begins in May of 2019. One may imagine that after then being 15 years in chronic pain with chronic insomnia, (between 2008-2014 mostly operating on 2 hours), I had been deeply depressed. And before the pain, since age seven, I had that certain ideation... You all know the one. Chronic pain only made it worse. After years of all that, with no methods of medicine or therapy working, having entered an existential crisis for other epistemological reasons, I decided that, still, instead of calling it quits, I needed to try something else. I needed to try something drastic—That which I had been always told not to try. So I took one tab of LSD.

I waited through first hour, looking for any signs it was working. Nothing. Not uncommon, as things really pick up around hour two anyway. But hour two rolled around, and while I was beginning to feel something I never had before, all I wanted to do was go lay down and take a nap about it. Not only have I had insomnia most of my life, I've never been one to just lay down and take a nap. But to my surprise that is what I did for my first peak experience of LSD; I took a cat nap. I awoke, I think about an hour later, feeling well rested, having had beautiful rainbow landscape dreams, and feeling as if I had only just started my experience. Second peak was life-changing, and I have written about it elsewhere. Needless to say I no longer had that ideation, and a certain weight had been lifted. I wouldn't take LSD again for another three years.

In May of 2022 I sought to experience LSD again, but this time I wanted to see if I could have the same experience as everyone else, or if I would want to fall asleep again. So I bought a larger supply in order to test various doses. I began with one tab, and sure enough at first peak I wanted to go take a nap. At this point I

did not know how much I was taking in actual micrograms, but I eventually had one of my largest supplies tested. Through trial and error that spring and summer I would find that I could consistently fall asleep on one dose, two doses made me drowsy, I would still lay down, but the quality of sleep was disturbed, and it wasn't as long. Three doses and beyond was the threshold to keep me awake the entire time. After a few times in late summer and into the Fall I began to notice that I had been having more good sleep nights than bad nights in various ways, whether it was sleeping through the night, or just better sleep quality regardless the amount of hours, or some other altered pattern. The most noticeable pattern as I analyzed my experience was that at times of higher doses the better quality sleep seemed to last for nearly two weeks before I would revert back to my usual insomnia ways. It wasn't always an easy pattern to fall into at first, because after I successfully stayed awake all day on several doses I would stay awake that same night, unable to sleep soundly. It wouldn't be until the next night when the results would begin. So I made a slight adjustment. I began taking doses late in the evening, usually around 10pm. This way I would be up all night, remain awake all the next day, falling asleep early and soundly that next night after the dose.

THE DATA

We can't make bricks without clay, right? Well, here's what I was attuned enough to gather along the way as someone brought up in the humanities, and not the hard sciences. I wrote down nearly every date and dose, but I didn't necessarily track how long between doses before I would actually notice reverting back to old sleep patterns. This is for several reasons. For one, I was just happy to sleep for once, and didn't consider that later on it may be of relevance. Number two I was also working out many other things in my life, which resulted in outputting creatively: writing 100,000 words and painting several dozen paintings, one of which is 4ft x 4ft and highly detailed; so sometimes I would dose sooner than

necessary for maintaining the quality sleep simply because I wanted to stay up all night working on projects, or doing various meditation practices. Because of this I discovered that I can take the same dose up to three days later and in lieu of my experience being diminished due to tolerance, it is amplified, as if I took a larger dose. Third, when I wasn't taking LSD I was either away on vacation or traveling for work, or trying various means such as Psilocybin, MDMA, S-MDA, DMT, and the combination of LSD and DMT. I didn't have the same pattern reset success with any other substance, so there wasn't anything to note in regards to sleep anyway. The LSD and DMT combination did, however, increase dream production for the few nights that followed. So the data is slightly skewed in that: I didn't always dose strictly for maintaining sleep schedule, meaning sometimes I ingested sooner than necessary; it wasn't consistent at times due to travel or other life circumstance; I randomly tried various other substances in place of LSD.

In December of 2023 I received a brand new supply in liquid form. I had it tested and it returned as 80 μ g per drop. I ingested anywhere from 80–1,200 μ g on a rolling average of five days, with an average of ~560 μ g per trip.

One drop at 80 μ g had me sleepy like one tab at unknown dose; 160 μ g didn't make me drowsy as much as it made my eyes heavy and my body restless; 240–320 μ g was fine enough, it did my sleep well, but it was poor for motivation and creativity, and brought a more flattened emotion too tired to explore rather than an excited or anticipatory emotion; 400–560 μ g was ideal all around, not too heavy in any aspect, best for sleep quality, reasonable for sanity; 640–720 μ g was a lot like 240–320 μ g but without the lethargy and flattened emotion; 800–1,200 μ g, while spectacular in their own right, are not necessary doses for maintaining quality sleep resets. Additionally, I'm already taking enough as it is.

Somewhere between October–December of 2024 I began to feel like I just really wasn't in the mood, or up to the task of taking the LSD anymore, but I

continued to do so to maintain the sleep, and because I was nearing the end of that supply anyway; my last dose was December 14. About three weeks had come and gone when I realized that I was still having more good sleep nights than bad, and then another week passed, and then another, until I reached 8 weeks of more quality sleep in a row than I can ever remember attaining in my life. I did break that streak, however, there is something that is beginning to feel different overall, and time during this next experimental phase will tell.

HYPOTHESIS

I am part of a population of patients who show evidence of low serum serotonin levels (Al-Nimer, M. S. M., et al 2018); low levels of serotonin may be responsible in part for chronic pain (Paredes, S., et al 2019; Häuser, W., et al 2013). I also shows classic signs of alterations in circadian cortisol rhythmicity found in patients with Chronic Insomnia (CI) (Vargas, I., et al 2018). The interactions in my personal study and qualitative analysis may be an indicative sign of an overshadowing deviant regulation of the hypothalamic–pituitary–adrenal (HPA) axis. This deviation could trickle down into the production of; low levels of serotonin, PBMC dysfunctions, abnormal levels or behaviors of cortisol, and other irregularities. It is hypothesized that LSD, with its unique receptor attachment behaviors (Wacker, D., et al 2017), acts as a stabilizer to receptor conformations which improve both endogenous serotonin substrate supplementation and receptor functionality for a short time in patients with decreased levels of serotonin and/or CI. While the association is bidirectional it is inconclusive whether a deviant HPA axis has a stronger impact on low serum serotonin levels than vice versa; regardless, I proposed that in both similarly affected groups and healthy groups serum tryptophan and serotonin levels are gathered accordingly to see if a dose of LSD could potentially disinhibit or modulate an otherwise dysfunctional serotonin pathway, temporarily mend sick or mutated receptors, or regulate a deviant HPA

axis. Additionally, cortisol pulsatility data could be beneficial for determining the effect of LSD on circadian and ultradian cortisol rhythms in patients reporting CI.

For further and better understanding of LSD response to circadian and ultradian rhythms it is proposed that further research be put into studying the effects of LSD on biological systems of low complexity such as dandelion inflorescences (Lehman, H.E., et al 1960), or as I would suggest, sunflower inflorescences; where panicles show their symmetrical time courses, capitulum offer a view into a dynamic rhythmicity that may be of much more value here.

CONCLUSION

My journey through the labyrinth of chronic pain and insomnia—marked by decades of failed treatments and unanswered questions—culminated in an unlikely conductor: LSD. What began as a desperate experiment revealed a profound truth—that consciousness-altering compounds might act as tuning forks for dysregulated biological systems, teaching me that perhaps psychedelics are neither medicines nor toxin, but conductors—momentarily synchronizing our discordant neural ensembles. This anecdotal data may suggest that LSD temporarily “overclocks” serotonin pathways and HPA axis dynamics, creating a window for neural reorganization. Lower doses induced sedation, while higher ones maintained wakefulness—a bidirectional effect revealing a contextual polarity which may indicate that LSD toggles between amplifying and dampening modes in neural networks. Additionally, the sustained benefits post-discontinuation suggest psychedelics might “teach” biological systems new attractor states. This aligns with fibromyalgia research implicating maladaptive plasticity in chronic pain.

The true mystery lies in the lingering effects—the eight weeks of restored sleep following discontinuation. This, a sort of informational hysteresis, where intense resonance leaves imprints that persist beyond the initial stimulus. Like a lake

retaining the memory of a stone's impact, my nervous system appears to have absorbed patterns of coherence from repeated psychedelic sessions.